



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ثريا فهم محمد البوهي
المحافظة : المنوفية
الحى / المدينة : اشمون
رقم البطاقة : 28606211700808 رقم التليفون : 01012783819
العنوان : اشمون
العمر (تاريخ الميلاد) : 21-06-1986 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 22-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ثريا فهد محمد البوحي National ID: 28606211700808	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

NAME: thoria Faheem mohamed

DATE: 19/7/2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.
- Small hypodense lesion seen at segment V measures 8mm likely cyst
- No intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected, IUCD is in place

Right inguinal enlarged suspicious LNs with thickened cortex 12mm for biopsy

Opinion:

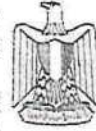
Hepatic focal lesion likely cyst

Best regards
Dr. NERMEEN RAMADAN FEBR, MD



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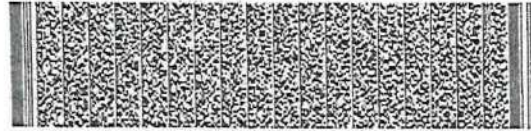
بنون عمل



أنثى مسلمة متزوجة
وليد عبد الباسط عبد الصمد برى

البطاقة مارية حتى ٢٠٣٢/١١/٢٥

١٢



جمهورية مصر العربية
بطاقة تحقيق الشخصية

ثريا

فهم محمد البوهي

عزبة لمعى

مركز اشمون - المنوفية

١٩٨٦/٠٦



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LM4390607

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٠١٠٩ - ٥٨٩٢ - ١

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female / male patient presented with diagnosis
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She/He has received (3D IMRT) radiotherapy to.....
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At Dose: 30 Gy/10 IN/2 w.

Started: 8.1.8/2026

Ended: 20/8/2026 With good tolerance.

Head of department
Dr. Chang William

Dr. [Signature]

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Sex/Age	Female 40 Y	Patient No.	25632
Registered Date	08/8/2026 02:22:12 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 01:17:48 PM	Branch	Gustave Roussy
Reporting Date	31/8/2026 01:24:41 PM	Pathology No.	2489/26

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Clinical Data: Right inguinal pathological lymph node in a patient with history of mixed ductal & lobular breast carcinoma.

Nature of Specimen: Tru-cut biopsy, collected on 08/08/2026.

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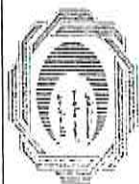
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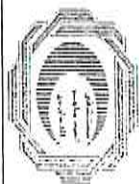
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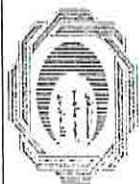
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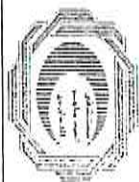
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(sequential / concomitant)

Started: 8/8/2026

Ended: 20/8/2026 With good tolerance .

Head of department
Dr. Hany William

5/8/26

Patient Name	ثريا محمد	Case No.	346803
Sex/Age	Female 40 Y	Patient No.	25632
Registered Date	08/8/2026 02:22:12 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 01:17:48 PM	Branch	Gustave Roussy
Reporting Date	31/8/2026 01:24:41 PM	Pathology No.	2489/26

Pathology Report

Clinical Data: Right inguinal pathological lymph node in a patient with history of mixed ductal & lobular breast carcinoma.

Nature of Specimen: Tru-cut biopsy, collected on 08/08/2026.

Gross Examination:

-Two fragmented grey, white tissue cores, ranging in size between 0.5 cm & 1.0 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- GATA-3 clone (L50-823), ER clone (1E2), PR clone (SP1) & Her2 clone (4B5).

Microscopic examination:

- Histopathological examination reveals cores of tumor formed of clusters of malignant epithelial cells showing enlarged moderately pleomorphic hyperchromatic nuclei with poor tendency to tubular formation amid desmoplastic stroma. No lymphoid tissue noted.

Results:

* IPT (All controls showed appropriate reactivity):

- GATA-3: Diffuse strong positive staining in tumor cells.
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 5% of tumor cells (score 5/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).

Conclusion:



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Right inguinal lymph node, Tru-cut biopsy:

- Consistent with metastatic carcinoma of breast origin.

IPT:

- ER: Positive (score 8/8).

- PR: Positive (score 5/8).

- Her2: Ultra-low (score 0+).

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



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